

AGEING NAVIGATOR

MVP PRODUCT PILOT & DEVELOPER BUILD BRIEF

Revised to Include Paid Specialist Tools — September 2026

1. PROJECT OVERVIEW

Ageing Navigator is an independent New Zealand ageing-navigation and transition service.

The core premise is:

Something has changed. What should happen next?

The trigger may include:

- dementia diagnosis;
- stroke;
- hospital admission or discharge;
- fall;
- increasing frailty;
- mobility problems;
- difficulty managing the home;
- loss or illness of a spouse/carer;
- increasing support needs;
- considering retirement living;
- considering residential care;
- or simply planning ahead.

The user often does not know what support they need or whether their current living situation remains appropriate.

Ageing Navigator helps them consider three primary pathways:

1. Stay at Home
2. Retirement Village
3. Residential Care

The core navigation service remains free.

The user receives a personalised Action Plan and, where appropriate, matching with relevant services, retirement villages or residential-care facilities.

Ageing Navigator has three commercial layers:

FREE NAVIGATION

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PAID SPECIALIST DECISION TOOLS

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PAID TRANSITION IMPLEMENTATION

The user can:

- implement the free Action Plan themselves;
- purchase a specialist \$99 digital tool where deeper financial analysis is useful;
- or ask Ageing Navigator to coordinate the transition for them as a paid service.

2. PURPOSE OF THE MVP

The MVP should test both the navigation model and the commercial model.

We need to learn:

- whether users complete the questionnaire;
- whether pathway recommendations make sense;
- whether the Action Plan is useful;
- whether provider/village matching is useful;
- whether users will pay for deeper specialist analysis;
- whether \$99 is an acceptable price;
- which paid tool generates the strongest demand;
- whether paid-tool customers subsequently request implementation;
- what information is missing;

- which implementation services users request;
- how much human time implementation requires;
- whether paid transition services can be delivered profitably;
- and which distribution sources generate paying customers.

The MVP must remain simple, flexible and inexpensive to modify.

3. PRODUCT ARCHITECTURE

LEVEL 1 — FREE AGEING NAVIGATOR

Helps answer:

What pathway should I explore?

Produces:

- personalised Action Plan;
- relevant funding/assessment information;
- support/service options;
- retirement-village matching;
- residential-care options;
- broad cost considerations where appropriate;
- next steps.

LEVEL 2 — PAID SPECIALIST TOOLS

Two initial tools:

TOOL 1 — VILLAGE & RESIDENTIAL CARE COST COMPARISON

Initial testing price:

NZ\$99

Core question:

What will the options I am considering actually cost me?

TOOL 2 — FUNDING ELIGIBILITY & PREPARATION TOOL

Initial testing price:

NZ\$99

Core question:

What government funding or support pathways appear relevant to my circumstances, and what should I do next?

LEVEL 3 — PAID IMPLEMENTATION

Three transition services:

1. Home Support Setup
2. Retirement Village Transition
3. Residential Care Search & Transition

4. CORE USER FLOW

The main flow should be:

TRIGGER / REASON FOR VISIT

↓

CONDITIONAL QUESTIONNAIRE

↓

RULES + VERIFIED KNOWLEDGE

↓

TRANSITION PATHWAY



DEEP PATHWAY QUESTIONS



FREE PERSONALISED ACTION PLAN



FREE MATCHING WHERE RELEVANT



RELEVANT PAID SPECIALIST TOOL OFFER



\$99 PAYMENT



DEEPER SPECIALIST QUESTIONS



PAID PERSONALISED REPORT



OPTIONAL PAID TRANSITION SUPPORT



FEEDBACK / TRACKING

A user does not need to purchase a \$99 tool before requesting Transition Support.

5. FIRST SCREEN — WHAT HAS CHANGED?

The questionnaire should initially establish why the user is seeking help.

Possible options:

- New dementia diagnosis
- Stroke
- Hospital admission or discharge
- Fall
- Increasing mobility problems
- Increasing support needs
- Managing the home is becoming difficult
- Spouse/carer can no longer provide the same support

- Considering retirement living
- Considering residential care
- Planning ahead
- I'm not sure what we need
- Other

The trigger does not automatically determine the final pathway.

6. PATHWAY 1 – STAY AT HOME

The user wants, or may be able, to remain at home with additional support.

Questions may consider:

- current living arrangement;
- mobility;
- personal care;
- household tasks;
- meals;
- shopping;
- transport;
- cleaning;
- gardening;
- medication support;
- loneliness/social support;
- cognition;
- falls/safety;
- family support;
- existing home care;
- respite;
- equipment;
- personal alarms;
- assessment status;
- funding concerns;
- approximate private budget where relevant.

FREE OUTPUT:

HOME SUPPORT PLAN

This may identify:

- support needs;
- priority actions;
- relevant funding/assessment pathways;
- suitable local services;
- provider options;
- indicative costs where reliable;
- questions to ask;
- what the family can organise themselves;
- next actions.

PAID IMPLEMENTATION:

HOME SUPPORT SETUP

Ageing Navigator coordinates the agreed support required to help the person remain at home.

7. PATHWAY 2 — RETIREMENT VILLAGE

This should remain one of the most detailed free pathways because village matching may become a major acquisition product.

Questions may include:

- preferred location;
- proximity to family;
- broad budget;
- accommodation type;
- villa/apartment preference;
- lifestyle;
- interests;
- social preferences;
- pets;

- transport;
- mobility;
- cognition;
- current support needs;
- expected future support needs;
- serviced living;
- care suites;
- higher care onsite;
- desire to avoid another future move;
- timeframe;
- other priorities.

8. FREE RETIREMENT VILLAGE MATCHING

The free product answers:

Which villages appear to suit me?

The matching should primarily consider:

- location;
- lifestyle;
- interests;
- accommodation preference;
- pets;
- proximity to family;
- mobility;
- current support;
- future support;
- future-care requirements;
- broad budget suitability.

It should not attempt to provide the detailed financial comparison offered by the paid tool.

The purpose is to reduce a large number of possible villages to a useful shortlist.

Example:

VILLAGE A

Why it matched:

- preferred location;
- villa option available;
- relevant lifestyle facilities;
- future care available onsite.

Something to consider:

- pet restrictions may apply.

Matching should explain WHY each option appears relevant.

The system should not simply provide a directory.

9. PATHWAY 3 — RESIDENTIAL CARE

This pathway is for users where higher levels of care are already required or may need to be explored.

Questions may include:

- formal assessment status;
- assessed care level where known;
- dementia/cognitive requirements;
- hospital/discharge situation;
- current living arrangement;
- geographic preference;
- proximity to family;
- accommodation preferences;
- funding position;
- private-pay position where relevant;
- Residential Care Subsidy status;
- Residential Care Loan status;

- urgency;
- family priorities.

The system must not independently determine formal residential-care level.

Where a professional assessment is required, the Action Plan must explain this and identify the appropriate next step.

10. "I'M NOT SURE" PATHWAY

Users should not need to know whether they belong in:

- Stay at Home;
- Retirement Village;
- Residential Care

before starting.

There should be an:

I'M NOT SURE WHAT WE NEED

option.

The questionnaire should ask broader questions and guide the user towards relevant pathways.

The system may recommend more than one option.

For example:

Stay at Home now while beginning to explore retirement living for the future.

11. CONDITIONAL QUESTIONNAIRE

The questionnaire should be dynamic.

Users should only see questions relevant to their circumstances.

Examples:

If dementia is selected:

→ ask relevant cognition, safety, support and future-planning questions.

If the user has a formal residential-care assessment:

→ ask assessed care level.

If no assessment has occurred:

→ do not ask questions that assume an assessment exists.

If retirement living is relevant:

→ ask detailed matching questions.

Users should be able to answer:

- Unsure
- Don't know
- Not applicable

where appropriate.

The system must not require users to guess information they do not know.

12. QUESTION CONFIGURATION

Questionnaire content should preferably be stored in structured configuration.

Possible fields:

- Question ID

- Pathway
- Question text
- Answer type
- Answer options
- Required/optional
- Show when
- Rule trigger
- Notes

A sophisticated questionnaire-builder platform is not required.

The objective is to allow wording and logic to be changed without major redevelopment.

13. DECISION RULES

Recommendations should be rules-led.

AI should not independently determine:

- required clinical care level;
- formal government eligibility;
- legal conclusions;
- financial advice;
- clinical conclusions.

Examples:

IF user needs household help but is managing personal care
→ consider Stay at Home support.

IF future care is important
→ prioritise villages with suitable future-care options.

IF formal residential-care assessment exists
→ use the assessed care level as a matching requirement.

IF professional assessment is required
→ clearly identify this in the Action Plan.

Rules should be editable without substantial redevelopment.

14. VERIFIED KNOWLEDGE REPOSITORY

The repository is a critical component.

Version 1 should initially focus on:

TAURANGA / BAY OF PLENTY

along with relevant national New Zealand information.

The repository may include:

- needs-assessment information;
- government funding;
- Residential Care Subsidy;
- Residential Care Loan;
- dementia support;
- home support;
- cleaning;
- gardening;
- transport;
- meals;
- mobility/equipment;
- personal alarms;
- retirement villages;
- residential-care providers;
- moving/downsizing;
- legal/EPOA resources;
- community support;
- relevant national organisations.

Important records should include:

- Record ID
- Topic
- Provider/organisation where applicable
- Region
- Applies when
- Approved explanation
- Services
- Contact information
- Website
- Source
- Source URL/document
- Last checked
- Review date/status
- Active/inactive

AI is not the source of truth.

15. SOURCE HIERARCHY

Where information changes over time, Ageing Navigator should use a source hierarchy.

Preferred order:

1. Official government sources
2. Health NZ / relevant health authorities
3. Official regional services
4. Official provider documents
5. Verified provider websites
6. Other reliable secondary sources where required

Every important rule or financial figure should remain traceable to its source.

Where information conflicts, it should be flagged for manual review rather than automatically resolved by AI.

16. PROVIDER REPOSITORY — HOME SUPPORT

Potential fields include:

- provider name;
- service type;
- area covered;
- pricing where available;
- minimum service;
- contact information;
- availability where known;
- source;
- date checked.

No national automated provider database is required for the MVP.

Manual research and data entry are acceptable.

17. PROVIDER REPOSITORY — RETIREMENT VILLAGES

The village repository should contain two distinct categories of information:

A. MATCHING INFORMATION

B. FINANCIAL INFORMATION

MATCHING INFORMATION may include:

- village name;
- location;
- accommodation types;
- villas;
- apartments;
- serviced apartments;
- care suites;

- pets;
- lifestyle;
- facilities;
- transport;
- social activities;
- current support options;
- rest-home care;
- hospital care;
- dementia care;
- future-care options;
- key entry information.

FINANCIAL INFORMATION should be structured separately.

Potential fields include:

- entry price / licence price;
- accommodation/unit type;
- weekly/monthly village fee;
- fee increase rules where documented;
- deferred management fee structure;
- DMF percentage;
- DMF accrual period;
- maximum DMF;
- capital gain treatment;
- capital loss treatment;
- refurbishment/reinstatement obligations;
- resale-related charges;
- operator deductions;
- administration fees;
- additional service charges;
- care-suite charging structure;
- exit/payment timing;
- other material financial terms.

Each financial field should link back to the relevant source document.

18. PROVIDER REPOSITORY — RESIDENTIAL CARE

Potential fields include:

- facility;
- location;
- care levels provided;
- dementia care;
- hospital-level care;
- premium accommodation information;
- standard charges where available;
- contact information;
- availability if manually checked;
- source;
- date verified.

19. FINANCIAL DOCUMENT REPOSITORY

The paid financial comparison requires a structured source-document layer.

Potential documents may include:

- Occupation Right Agreement information;
- Disclosure Statements;
- village financial summaries;
- provider pricing information;
- official residential-care policies;
- government funding documents;
- Residential Care Subsidy information;
- Residential Care Loan information;
- Maximum Contribution information;
- premium accommodation information;
- other relevant policy documents.

The system should ideally work as:

SOURCE DOCUMENT

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EXTRACTED STRUCTURED FIELDS

↓

VERIFIED DATA

↓

CALCULATION RULES

↓

CUSTOMER REPORT

The original source should remain traceable.

AI may assist with extracting information from documents.

However, financial facts used for calculations must be verified before being treated as approved data.

20. FREE MATCHING ENGINE

Matching should initially be rules based.

The objective is not to create a mathematically perfect ranking.

The objective is to produce a useful shortlist and explain why each option may fit.

For example:

VILLAGE A

Why it matched:

- preferred area;
- preferred accommodation type;

- pet policy appears suitable;
- future-care options available.

Something to consider:

- limited transport options.

Commercial relationships must not determine matching or ranking.

Ageing Navigator should not accept payment from providers to rank higher.

21. PAID TOOL 1 — VILLAGE & RESIDENTIAL CARE COST COMPARISON

INITIAL TESTING PRICE:

NZ\$99

The price should be configurable without redevelopment.

CORE QUESTION:

What will the options I am considering actually cost me?

This product is deliberately different from the free village-matching tool.

FREE TOOL:

Which villages or care options appear to fit my circumstances?

PAID TOOL:

What are the financial implications of the options I am seriously considering?

The user may select options from the free shortlist or choose other options where sufficient verified information exists.

22. PAID COST COMPARISON — CUSTOMER INFORMATION

Only information genuinely required to produce a meaningful comparison should be collected.

Potential inputs may include:

- single/couple;
- approximate property value;
- mortgage/debt where relevant;
- available capital range;
- savings/assets where relevant;
- planned retirement-village budget;
- expected timeframe;
- whether a partner remains at home;
- funding situation where relevant;
- relevant care arrangement;
- scenario duration.

Financial ranges should be used where exact figures are unnecessary.

23. PAID COST COMPARISON — FINANCIAL CALCULATIONS

Depending on available verified information, the tool may calculate or compare:

- initial capital required;
- entry price;
- ongoing village fees;
- deferred management fee;
- estimated exit deductions;
- other documented deductions;
- capital gain treatment;
- capital loss treatment;

- refurbishment/reinstatement costs;
- resale-related charges;
- premium accommodation costs;
- residential-care charges;
- relevant government contributions;
- estimated total cost over time;
- estimated capital returned on exit.

Potential comparison periods:

3 YEARS

5 YEARS

10 YEARS

The tool should only calculate what can be supported by verified information.

Where important information cannot be confirmed, show:

NOT CONFIRMED

rather than inventing an estimate.

24. PAID COST COMPARISON — REPORT

The customer should receive a professional:

PERSONALISED VILLAGE & CARE COST COMPARISON

Potential sections:

A. YOUR SITUATION

Relevant financial and living circumstances.

B. OPTIONS COMPARED

Selected villages or care options.

C. UPFRONT COSTS

Side-by-side comparison.

D. ONGOING COSTS

Comparable recurring costs.

E. EXIT / DEFERRED FEES

Explain major financial consequences.

F. COST OVER TIME

Where sufficient data exists:

- 3-year scenario;
- 5-year scenario;
- 10-year scenario.

G. CARE COST CONSIDERATIONS

Where relevant.

H. IMPORTANT DIFFERENCES

Plain-English explanation of differences between options.

I. INFORMATION STILL TO CONFIRM

Anything not adequately verified.

J. QUESTIONS TO ASK

Important questions the customer should clarify before making a decision.

K. NEXT STEPS

Including relevant Ageing Navigator Transition Support.

The system must not provide legal, investment or financial advice.

It provides factual comparison, calculations and decision-support information.

25. PAID TOOL 2 — FUNDING ELIGIBILITY & PREPARATION

INITIAL TESTING PRICE:

NZ\$99

The price should be configurable.

CORE QUESTION:

What government funding or support may apply to my circumstances, and what should I do next?

Potential pathways may include:

- needs-assessment related support;
- funded home support;
- Residential Care Subsidy;
- Residential Care Loan;
- other verified ageing-related government support.

The scope should only expand where sufficient verified rules and information are available.

26. FUNDING TOOL — CUSTOMER INFORMATION

Questions may include:

- age;
- residency status where relevant;
- relationship status;
- living arrangement;
- home ownership;
- whether a partner remains at home;
- asset ranges;
- savings/investment ranges;
- income ranges;
- assessment status;
- current care arrangement;
- expected pathway.

The questions must follow the actual requirements of the relevant official funding rules.

27. FUNDING TOOL — REPORT

The user receives:

PERSONALISED FUNDING & SUPPORT REPORT

Potential sections:

A. YOUR CIRCUMSTANCES

Plain-English summary.

B. FUNDING PATHWAYS WORTH EXPLORING

Based on the information supplied and verified rules.

C. WHY THEY MAY BE RELEVANT

Relevant criteria explained in plain English.

D. WHAT STILL NEEDS TO BE CONFIRMED

Unknown or missing information.

E. DOCUMENTS TO PREPARE

Relevant checklist.

F. WHO MAKES THE DECISION

Clearly state the relevant authority.

G. APPLICATION / ASSESSMENT PROCESS

Step-by-step guidance.

H. QUESTIONS TO ASK

Useful preparation.

I. NEXT STEPS

Priority actions.

J. IMPLEMENTATION SUPPORT

Option for Ageing Navigator to coordinate relevant next steps.

The report should use wording such as:

Based on the information you have provided, this funding pathway appears potentially relevant.

It must not state that a customer has been formally approved or definitely qualifies unless that decision comes from the authorised organisation.

28. FREE VS PAID BOUNDARY

The distinction must be clear.

FREE AGEING NAVIGATOR

Should provide enough value to be useful on its own.

This may include:

- likely pathway;
- broad funding pathways;
- service/village matches;
- broad cost considerations;
- priority next steps.

\$99 SPECIALIST TOOLS

Provide deeper analysis requiring:

- additional questions;
- structured financial information;
- detailed calculations;
- personalised scenario comparison;
- deeper funding analysis;
- preparation information.

The paid product must not simply hide information that should reasonably be part of the free Action Plan.

It should provide materially greater decision-making value.

29. PAYMENT FUNCTIONALITY

Version 1 now requires simple payment functionality for the two specialist tools.

Requirements:

- one-off NZ\$99 payment;
- secure third-party payment gateway;
- successful payment confirmation;
- payment linked to case/order ID;
- access to specialist questionnaire/report after payment;
- receipt/confirmation;
- admin visibility of payment status.

Do NOT build:

- subscriptions;
- wallets;
- provider payments;
- payment splitting;
- complex invoicing.

Paid Transition Services can initially be invoiced and handled manually outside the application.

30. FREE ACTION PLAN

The Action Plan remains the main free output.

It should be generated from:

- questionnaire answers;
- rules;
- verified repository data;
- provider matches;
- controlled AI summarisation.

Suggested sections:

A. YOUR CURRENT SITUATION

B. WHAT MATTERS MOST

C. YOUR LIKELY PATHWAY

D. YOUR PRIORITY ACTIONS

Each action should ideally include:

- What to do
- Why it matters
- Next step
- Who can help
- What to prepare
- Questions to ask

E. FUNDING / ASSESSMENT INFORMATION

F. SUPPORT OR PROVIDER MATCHES

G. THINGS TO CHECK

H. PROFESSIONAL ASSESSMENT / ADVICE

I. RELEVANT SPECIALIST TOOL

J. PAID IMPLEMENTATION OPTION

31. REPORT FORMAT

The free Action Plan should be:

1. viewable online; and
2. available as a professional PDF.

Paid specialist reports should also be:

- viewable online;
- available as professional PDFs;
- consistently branded;
- suitable for sharing with family or professional advisers.

The design should remain consistent regardless of the amount of personalised content.

32. AI USE

AI may assist with:

- summarising questionnaire responses;
- converting structured information into plain English;
- drafting Action Plan explanations;
- summarising provider information;
- source-document extraction assistance;
- drafting financial comparison explanations;
- preparing clear next steps;
- drafting funding explanations.

AI must NOT be the source of truth.

AI must not invent:

- financial thresholds;
- funding eligibility;
- provider pricing;
- village fees;
- ORA terms;
- government processes;
- care levels;
- clinical conclusions;
- legal conclusions;
- availability.

Important factual information must come from:

RULES

+

VERIFIED REPOSITORY DATA

+

APPROVED SOURCE DOCUMENTS

33. EMAIL CAPTURE

Users may need to provide an email address to receive:

- complete Action Plan;
- PDF;
- complete village/provider shortlist;
- paid specialist report;
- payment receipt.

Suggested fields:

- Name
- Email
- Phone optional
- Consent to receive report

- Separate marketing-consent checkbox

Email required for report delivery must not automatically enrol someone into marketing.

34. PAID TRANSITION SERVICE 1 – HOME SUPPORT SETUP

Purpose:

Coordinate agreed support so the person can remain at home.

Possible areas include:

- home care;
- cleaning;
- gardening;
- meals;
- transport;
- alarms;
- equipment;
- other practical services;
- relevant assessment/funding coordination.

Needs assessment and funding are not separate implementation products.

Where required to establish home support, Ageing Navigator may coordinate:

- assessment enquiries;
- appointment organisation;
- preparation;
- relevant paperwork;
- follow-up.

Specialist organisations still make formal decisions.

The objective is to sell an outcome rather than hours.

35. PAID TRANSITION SERVICE 2 — RETIREMENT VILLAGE TRANSITION

The service starts before the customer has necessarily chosen a village.

Potential stages:

STAGE A — MATCH & CHOOSE

Ageing Navigator may coordinate:

- detailed requirements;
- shortlist;
- village enquiries;
- information gathering;
- appointments;
- tours;
- comparison;
- family questions;
- future-care considerations;
- relevant assessment process where required.

STAGE B — MOVE & TRANSITION

Once a village is selected, Ageing Navigator may coordinate:

- village liaison;
- transition timetable;
- solicitor/ORR preparation;
- downsizing;
- decluttering;
- movers;
- storage;
- cleaning;
- gardening;

- property preparation;
- utilities;
- address changes;
- support services;
- agreed move-day tasks;
- settling-in support.

Specialist providers perform specialist work.

Ageing Navigator coordinates it.

36. PAID TRANSITION SERVICE 3 — RESIDENTIAL CARE SEARCH & TRANSITION

Purpose:

Coordinate the search and transition into suitable residential care.

Possible tasks include:

- confirming assessment status;
- confirming assessed care level where available;
- clarifying family requirements;
- shortlist creation;
- contacting facilities;
- checking availability;
- obtaining information;
- arranging discussions/visits;
- preparing questions;
- comparing options;
- funding-process preparation;
- facility liaison;
- transition planning;
- moving coordination;
- agreed property/home tasks.

Ageing Navigator does not make the clinical determination of care level.

37. IMPLEMENTATION REQUEST

At the appropriate point, show a CTA relevant to the pathway.

Examples:

Would you like Ageing Navigator to organise your Home Support Setup?

Would you like Ageing Navigator to help coordinate your Retirement Village Transition?

Would you like Ageing Navigator to search for and coordinate Residential Care?

Paid specialist-tool customers should also receive relevant implementation CTAs.

38. CROSS-SELLING

Where genuinely relevant, the system may offer:

FREE NAVIGATOR

→ VILLAGE & CARE COST COMPARISON

FREE NAVIGATOR

→ FUNDING TOOL

VILLAGE & CARE COST COMPARISON

→ FUNDING TOOL

FUNDING TOOL

→ COST COMPARISON

EITHER PAID TOOL

→ TRANSITION IMPLEMENTATION

Cross-selling should only occur where relevant to the person's situation.

39. IMPLEMENTATION TRACKING

The admin system should allow Ageing Navigator to track:

- implementation requested;
- service type;
- originating pathway;
- specialist tool purchased;
- tasks requested;
- Navigator assigned manually;
- total Navigator time;
- organisations contacted;
- calls/emails;
- barriers;
- outcome;
- customer feedback;
- estimated delivery cost;
- eventual price;
- gross contribution;
- notes.

A sophisticated case-management platform is not required initially.

40. PAID TOOL ANALYTICS

For each specialist tool track:

- tool shown;
- offer viewed;
- checkout started;
- payment completed;

- questionnaire started;
- questionnaire completed;
- report generated;
- abandonment point;
- refund/support request;
- other tool purchased;
- implementation requested;
- implementation purchased;
- referral source.

The key questions are:

Does the \$99 tool generate profitable direct revenue?

AND:

Does purchasing the \$99 tool increase the likelihood of purchasing Transition Support?

41. FEEDBACK

FREE ACTION PLAN FEEDBACK:

- How useful was the plan?
- Did the recommendations make sense?
- Was anything important missing?
- Was anything confusing or incorrect?
- Would you like Ageing Navigator to organise any next steps?

PAID TOOL FEEDBACK:

- Was the report worth the price?
- Did it make your decision easier?
- Was important information missing?
- Was anything difficult to understand?
- Did the report change what you planned to do?

- Would you like Ageing Navigator to help implement the next steps?

Optional free-text feedback should be available.

42. REFERRAL SOURCE TRACKING

Distribution is a major part of the pilot.

The system should capture referral/acquisition source.

Example:

ageingnavigator.co.nz/start?ref=source

Potential sources include:

- Home Carers carer
- GP clinic
- financial adviser
- lawyer
- retirement village
- hospital/social worker
- community organisation
- cleaning company
- gardening company
- flyer/QR code
- Google organic
- Google Ads
- Facebook
- Home Carers website
- direct
- other

The referral source should remain attached to the customer through:

FREE NAVIGATOR

→ PAID TOOL

→ IMPLEMENTATION

This allows Ageing Navigator to calculate revenue by acquisition source.

43. ADMIN REQUIREMENTS

The MVP requires a simple secure admin area.

Admin should be able to:

- view cases;
- view questionnaire answers;
- view pathways;
- view generated Action Plans;
- view provider matches;
- manage repository records;
- manage village financial information;
- manage funding information;
- access source references/documents;
- update verification status;
- view paid-tool purchases;
- view payment status;
- view paid reports;
- view referral source;
- view feedback;
- view implementation requests;
- update common questionnaire content where practical;
- export data to CSV.

Do not build a complex CRM.

44. PILOT RECORD

Internally retain enough information to evaluate each case.

Suggested data:

- Case ID
- Date
- Contact details if supplied
- Trigger
- Questionnaire answers
- Pathway
- Provider matches
- Free Action Plan
- Specialist tool offered
- Tool purchased
- Payment status
- Specialist answers
- Paid report
- Referral source
- Feedback
- Implementation request
- Implementation outcome

There is no requirement for users to maintain a persistent account in Version 1.

45. PRIVACY AND SECURITY

The product may contain sensitive personal and financial information.

Minimum requirements:

- HTTPS;
- reputable managed hosting;
- secure database;
- secure admin authentication;

- minimal unnecessary data collection;
- appropriate consent;
- no user able to view another user's data;
- secure AI/API handling;
- family information not used to train external AI models;
- financial ranges used instead of exact values where sufficient;
- data retained only where required for legitimate business purposes.

The developer should identify any additional privacy/security issues before launch.

46. TECHNICAL APPROACH

The MVP should prioritise:

- speed;
- reliability;
- low operating cost;
- maintainability;
- easy modification;
- mobile-friendly design;
- future extensibility.

The application now requires:

- responsive web frontend;
- conditional questionnaire;
- structured question configuration;
- rules engine;
- verified knowledge repository;
- provider repository;
- financial data repository;
- source-document traceability;
- controlled AI integration;
- free matching engine;
- financial calculation engine;

- free Action Plan;
- paid specialist reports;
- PDF generation;
- email delivery;
- one-off payment gateway;
- feedback;
- implementation request;
- referral tracking;
- simple admin;
- data export.

47. DO NOT BUILD IN VERSION 1

Do not build unless specifically agreed:

- persistent Family Ageing Record;
- user accounts;
- family portal;
- multi-user family access;
- native iOS/Android apps;
- provider portal;
- provider login;
- provider dashboard;
- Navigator workforce platform;
- automated case allocation;
- complex CRM;
- live national vacancy database;
- automated provider booking;
- real-time provider integrations;
- provider marketplace;
- provider commissions;
- provider payment system;
- subscriptions;
- white-labelled widgets;
- public API;

- enterprise functionality;
- automated government applications;
- complex analytics dashboard;
- international functionality;
- autonomous AI agents;
- sophisticated machine-learning recommendation systems.

IMPORTANT:

A simple one-off customer payment system IS now required for the two \$99 specialist tools.

48. INITIAL GEOGRAPHIC SCOPE

The first pilot should focus primarily on:

TAURANGA / BAY OF PLENTY

National government information may still be included where applicable.

The objective is:

REGIONAL DEPTH BEFORE NATIONAL BREADTH

The repository should contain enough Tauranga information to make real customer plans and reports useful.

Dee can support cases remotely by phone and online.

Local attendance can initially be handled in Tauranga where required.

Local attendance should ultimately be treated as an optional paid service rather than required for standard delivery wherever possible.

49. PILOT USER MIX

Where possible, the initial pilot should include a mixture of:

- Stay at Home cases;
- Retirement Village cases;
- Residential Care cases;
- users unsure what they need;
- people relevant to the Village & Care Cost Comparison;
- people potentially eligible for funding/support;
- users likely to request implementation.

Triggers may include:

- dementia;
- stroke;
- hospital discharge;
- falls;
- mobility changes;
- increasing support needs;
- planning ahead.

These are triggers rather than separate products.

50. PILOT METRICS

USER BEHAVIOUR

- Questionnaire started
- Questionnaire completed
- Drop-off point
- Pathway
- Unknown/unsure answers

ACTION PLAN

- Usefulness
- Accuracy
- Missing information
- Matching relevance

PAID SPECIALIST TOOLS

- Offer shown
- Checkout started
- Purchases
- Conversion rate
- Revenue
- Completion rate
- Report usefulness
- Refund rate
- Implementation conversion

IMPLEMENTATION

- Implementation requested
- Which package
- Tasks requested
- Willingness to pay
- Navigator time
- Delivery cost
- Gross margin

DISTRIBUTION

- Referral/acquisition source
- Users by source
- Completed plans by source
- Paid-tool sales by source
- Implementation sales by source
- Revenue by source

REPOSITORY

- Missing information
- Outdated information
- Missing financial documents
- Rules needing clarification
- New provider categories required

51. MVP DELIVERABLES

The developer should provide:

1. Mobile-responsive web application
2. Trigger/entry selection
3. Three core transition pathways
4. "I'm not sure" pathway
5. Conditional questionnaire
6. Editable question configuration
7. Basic rules engine
8. Knowledge repository
9. Provider repository
10. Financial/source-document repository
11. Free Retirement Village matching
12. Home-support matching
13. Residential-care matching
14. Controlled AI integration
15. Personalised free web Action Plan
16. Free PDF Action Plan
17. Village & Residential Care Cost Comparison Tool
18. Financial calculation engine
19. Paid cost-comparison report
20. Paid cost-comparison PDF
21. Funding Eligibility & Preparation Tool
22. Paid funding report
23. Paid funding PDF

24. One-off payment integration
25. Email delivery
26. Feedback system
27. Implementation request form
28. Referral-code tracking
29. Simple admin area
30. Paid-order/payment tracking
31. CSV/data export
32. Deployment
33. Source-code ownership and handover
34. Basic technical documentation

52. DEVELOPMENT MILESTONES

MILESTONE 1

Confirm revised scope, data model and commercial user flow.

MILESTONE 2

Clickable UX/prototype.

MILESTONE 3

Conditional questionnaire and rules architecture.

MILESTONE 4

Build the Retirement Village pathway end-to-end first.

This should be the first complete pathway because it supports both:

- free village matching; and
- the paid financial comparison tool.

MILESTONE 5

Build provider repository and free village matching.

MILESTONE 6

Build free Action Plan, controlled AI and PDF generation.

MILESTONE 7

Build financial repository and source-document structure.

MILESTONE 8

Build Village & Residential Care Cost Comparison.

MILESTONE 9

Build Funding Eligibility & Preparation Tool.

MILESTONE 10

Add payment gateway.

MILESTONE 11

Add Stay at Home and Residential Care pathways.

MILESTONE 12

Add feedback, referral tracking and implementation requests.

MILESTONE 13

Build basic admin and export.

MILESTONE 14

Test using dummy families and real policy/provider documents.

MILESTONE 15

Fix issues and launch Tauranga pilot.

53. ACCEPTANCE CRITERIA

The MVP is ready for pilot when:

- it works well on mobile;
- users can explain what prompted their visit;
- questions adapt based on answers;
- users can answer Unsure;
- users can be guided towards relevant pathways;
- retirement-village users receive meaningful free matching;
- Stay at Home users receive relevant support recommendations;
- Residential Care users receive appropriate options without unauthorised clinical decisions;
- the free Action Plan is coherent and personalised;
- factual information comes from rules/repository data;
- AI is constrained;
- source documents can be linked to financial data;
- financial fields can be updated;
- calculations trace back to approved information;
- users can securely pay \$99;
- paid reports unlock correctly;
- financial comparisons show understandable side-by-side information;
- funding reports explain potentially relevant pathways without claiming formal eligibility;
- web/PDF reports work consistently;
- email delivery works;
- implementation can be requested;
- referral source persists through the funnel;
- feedback is captured;

- admin can review cases, reports and payments;
- pilot data can be exported.

54. CONTENT SUPPLIED BY AGEING NAVIGATOR

Ageing Navigator will progressively provide:

- questionnaire wording;
- answer options;
- branching logic;
- pathway rules;
- government information;
- funding policies;
- needs-assessment information;
- provider information;
- village information;
- residential-care information;
- financial policy documents;
- Occupation Right Agreement/disclosure information where available;
- financial calculation requirements;
- Action Plan wording;
- paid-report wording;
- implementation-package definitions;
- feedback questions;
- brand assets;
- privacy wording;
- disclaimer wording.

The developer is responsible for building the system that allows this information to be used and updated.

55. VERSION 2 — ONLY IF THE PILOT JUSTIFIES IT

Potential future functionality may include:

- user accounts;
- Family Ageing Record;
- saved comparisons;
- updated reports over time;
- automated follow-up;
- email nurture;
- provider self-service updates;
- Navigator workflow system;
- automated provider enquiries;
- availability requests;
- recurring payments/subscriptions;
- wider regional rollout;
- white-labelled provider widgets;
- public/private APIs;
- CRM integrations;
- provider analytics;
- partner dashboards;
- richer sector insights;
- increased safe automation.

56. COMMERCIAL FUNNEL

The commercial journey should now be:

FREE NAVIGATION

Help me understand what I should do.



FREE MATCHING

Help me find options that appear to suit me.

↓

PAID SPECIALIST TOOL — \$99

Help me understand what these options will really cost.

AND/OR

Help me understand which government funding/support pathways may apply.

↓

PAID IMPLEMENTATION

Help me organise and coordinate the next step.

57. CORE PRODUCT PRINCIPLE

The free product must provide genuine value.

The paid digital tools must provide materially deeper analysis.

The paid Transition Services must solve the practical problem of:

Who is going to organise all of this?

58. FINAL PRODUCT SUMMARY

Ageing Navigator should not initially become a large aged-care software platform.

The MVP has four jobs:

1. Help families understand what is happening.

2. Help them identify the most relevant pathway.
3. Give them useful independent information and matching.
4. Test whether they will pay for deeper analysis or for Ageing Navigator to coordinate the transition.

The business model is:

FREE NAVIGATION

+

\$99 SPECIALIST DECISION TOOLS

+

PAID TRANSITION IMPLEMENTATION

The key distinction is:

FREE:

Help me understand my options and find suitable choices.

\$99:

Help me understand the financial implications or funding pathways in much greater detail.

PAID IMPLEMENTATION:

Help me make it happen.

The free and paid layers should share the same:

- questionnaire architecture;
- rules engine;
- verified repository;
- provider repository;
- AI controls;

- source-document structure;
- report-generation system.

They should not be built as separate standalone applications.

The first priority should be the Retirement Village pathway because it allows Ageing Navigator to test:

- free matching;
- financial repository data;
- the first \$99 specialist tool;
- paid digital conversion;
- and Retirement Village Transition enquiries

from the same underlying pathway.

The Tauranga pilot should then determine what is expanded, changed, automated or removed before Ageing Navigator scales nationally.